

Standard Operating Procedure

Managing Bed Non-Availability and Temporary Holding

«Hospital Name» — Supports AAC.2.d

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By	Quality Coordinator Name: _____ Signature: _____		
Approved By	Medical Superintendent Name: _____ Signature: _____		

1. Purpose

This SOP describes how «Hospital Name» manages patients when a bed is not available in the desired category, including temporary holding and redirection to alternate organisations.

2. Scope

Applies to every ward, ICU, day-care, and admission point where a bed-category or bed-availability mismatch occurs.

3. Responsibility

Bed Management / Nursing Administration: tracks bed availability and manages temporary holding placements.

Billing Desk: explains financial implications when the desired bed category is unavailable.

Medical Superintendent: is informed if a patient exceeds the defined temporary-holding time limit.

4. Procedure

1. When the desired bed category is unavailable, confirm current bed status by contacting the in-charge of each applicable ward directly, or by checking the hospital's defined bed-tracking mechanism (register, whiteboard, or bed-management software) if one exists — do not assume a bed is unavailable without this check.
2. If placed in temporary holding, confirm the holding area has adequate infrastructure to care for the patient safely, and log the arrival time in the Temporary Holding Log.
3. Explain the financial implications of the alternate bed category to the patient or family before confirming placement — quote the actual difference in daily bed charges from the hospital's current tariff list — and document that this explanation was given in the Bed-Category Mismatch Financial Explanation Record.
4. Do not exceed 4 hours in temporary holding without a decision to transfer the patient to an appropriate bed or place them per the Alternate Organisations List. This 4-hour limit is the organisation's adopted maximum, based on the source in Section 8 below. [Hospital to define: state here if a different maximum applies to a specific unit, e.g. ICU step-down, and why.]
5. If the 4-hour limit is reached, escalate immediately — call the hospital's on-duty administrator first; if unavailable, call the Medical Superintendent directly; if still unresolved, escalate to the CEO's office. [Hospital to define: enter the actual duty phone numbers for on-duty administrator, Medical Superintendent, and CEO's office here, so staff are not searching for a number during an active escalation.] Log the decision time and outcome in the Temporary Holding Log.
6. Keep the Alternate Organisations List current — verify contact details and available services at least once a month.

5. Records

Supported by Records 10–13: the Bed Non-Availability Guidance Log, Alternate Organisations List, Temporary Holding Log, and Bed-Category Mismatch Financial Explanation Record.

6. Related Documents

- HCO_AAC_2_v2_REWRITE_DRAFT — AAC.2 policy

7. Review

Reviewed annually, or sooner if bed capacity or the temporary-holding time limit changes.

8. References

- The Joint Commission. (n.d.). Leadership standards: Patient flow (Standard LD.04.03.11). <https://www.jointcommission.org/en-us/standards> (Official page; full standard text requires purchase or subscription.)